FAX

Pacific Crest Medical Center

4100 Harbor Mesa Parkway, Orange County, CA
Case Management Department

To	Illumination Health + Home, Recuperative Care Referrals
Fax	888-555-0155
From	Karen Lindqvist, RN, CCM
Phone	714-555-0143
Date	08/06/2026 15:42
Re	Recuperative care referral

MESSAGE

Referral for recuperative care. 54-year-old man with left leg cellulitis, now on oral antibiotics and medically ready for discharge 08/07. Unhoused, no place to elevate the leg or keep it clean. Independent with all care. Please review and advise.

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Recuperative Care Referral Form

Illumination Health + Home. Submit this form with supporting documents by fax to 888-555-0155.

REFERRING PARTY

Referring Hospital	Pacific Crest Medical Center	**Dept/Floor**	4 West Medical-Surgical
SW/CM/RN	Karen Lindqvist	**Phone/Pager #**	714-555-0143
Email	klindqvist@pacificcrest.example.org	**Nursing Station #**	714-555-0100
Authorized by	Priya Nandakumar, MD	**Phone #**	714-555-0100

PATIENT INFORMATION

Patient Name	MARCUS ELLERY	**MRN**	SYN-104417
Date of Birth	02/11/1972	**Ethnicity**	White
Insurance	Medi-Cal managed care plan	**Preferred Language**	English

Gender: [X] Male [ ] Female [ ] Transgender [ ] Other Last 4 # of Social Security: xxx-xx-0000

Housing Situation: [X] Currently Unhoused [ ] 4+ episodes of homelessness in the past 3 years

HOSPITAL ADMISSION DETAILS

Chief Complaint / Admitting Diagnosis: Cellulitis of left lower leg

Admit Date: 08/03/2026 Any Wounds? [X] Yes [ ] No

If yes, explain the number, location, size and stage: Healing 2 cm abrasion, left shin. No dressing needed

Independent with Wound Care? [X] Yes [ ] No If no, will Home Health be ordered? [ ] Yes [ ] No

Was PPD/TB Test or Chest X-Ray Performed? [ ] Yes [X] No Date: Results:

HISTORY OF HEALTH & LIVING SITUATION

O2	[ ] Yes [X] No	Active Infection	[X] Yes [ ] No
Self Administer All Meds	[X] Yes [ ] No	Ambulatory	[X] Yes [ ] No
Continent of Bowel & Bladder	[X] Yes [ ] No	Assistive Device	[ ] Yes [X] No
Colostomy/Ileostomy	[ ] Yes [X] No	Hospice	[ ] Yes [X] No
Foley Catheter	[ ] Yes [X] No	Car	[ ] Yes [X] No
Diabetes	[ ] Yes [X] No	Service Animal	[ ] Yes [X] No
Insulin	[ ] Yes [ ] No		
Diabetes related Oral Meds	[ ] Yes [ ] No		

Explain (O2 / infection / device / hospice): Left leg cellulitis, improving, on oral antibiotics

BEHAVIORAL AND MENTAL HEALTH STATUS

Does the patient have any limitations, behavioral or mental health challenges? [ ] Yes [X] No

[ ] Mental Health DX If checked, what type?

[ ] Non-compliant [ ] Forgetful [ ] Cognitive Impairment [ ] Registered Sex Offender

[ ] Other Explain:

Length of Stay Authorized in Recuperative Care: 30 Days

ILLUMINATION STAFF ONLY

Date Received: _____ Reviewed by: _____

Approved: [ ] Yes [ ] No Date: _____ Authorization #: _____ Reason for Denial: _____

Admission Date: _____ Admission Time: _____ Exit Date: _____

SYNTHETIC RECORD FOR A HACKATHON - NOT A REAL PATIENT - Claude Impact Lab OC 2026

Patient Face Sheet

Printed 08/06/2026 15:42 by Karen Lindqvist

PATIENT INFORMATION

Patient	ELLERY, MARCUS	**Sex**	M
DOB	02/11/1972	**Age**	54y
Address	NO FIXED ADDRESS	**Language**	English
Phone	NONE	**Marital Status**	Single
Emergency Contact	None listed	**Interpreter**	No

INSURANCE INFORMATION

Primary	Medi-Cal managed care plan	**Member ID**	SYN-M-552019
Secondary	None	**Financial Class**	Medi-Cal Managed Care

ENCOUNTER INFORMATION

Admit Date/Time	08/03/2026 21:17	**Mode of Arrival**	Walk-in
Patient Class	Inpatient	**Unit**	4 West Medical-Surgical
Admitting Diagnosis	Cellulitis of left lower leg	**Diagnosis Code**	L03.116, Z59.02
Attending	Priya Nandakumar, MD	**Expected Discharge**	08/07/2026

Pacific Crest Medical Center, 4100 Harbor Mesa Parkway, Orange County, CA. Main 714-555-0100.

ELLERY, MARCUS (MRN: SYN-104417) DOB: 02/11/1972 54y M Pacific Crest Medical Center · 4 West Medical-Surgical

Authorization Request: Recuperative Care (Community Support)

Submitted 08/06/2026 15:42 by Karen Lindqvist

Health Plan	Medi-Cal managed care plan
Member ID	SYN-M-552019
Service Requested	Recuperative care (medical respite)
Requested Provider	Illumination Health + Home
Days Requested	30
Diagnosis Code(s)	L03.116, Z59.02
Status	Pending health plan review
Authorization #	Not yet assigned

CLINICAL JUSTIFICATION

Referral for recuperative care. 54-year-old man with left leg cellulitis, now on oral antibiotics and medically ready for discharge 08/07. Unhoused, no place to elevate the leg or keep it clean. Independent with all care. Please review and advise.

History and Physical

Priya Nandakumar, MD - Admission date 08/03/2026 - Signed

CHIEF COMPLAINT

Redness, swelling and pain of the left lower leg for 3 days.

HISTORY OF PRESENT ILLNESS

54-year-old man with hypertension who scraped his left shin on a fence about one week ago. Over the last 3 days he developed spreading redness, warmth and pain from the ankle to mid-calf, with chills. Temperature 101.1 F in the emergency department. No drainage, no prior similar episodes.

Bedside ultrasound showed no fluid collection. Venous duplex negative for deep vein thrombosis. Blood cultures drawn. Started IV cefazolin and admitted for IV antibiotics and leg elevation.

PAST MEDICAL HISTORY

- Hypertension
- Tobacco use, half a pack per day

PAST SURGICAL HISTORY

None.

HOME MEDICATIONS

- Lisinopril 10 mg by mouth daily (takes inconsistently)

ALLERGIES

No known drug allergies.

SOCIAL HISTORY

Unhoused for about 14 months. Sleeps outdoors. Works occasional day labor. Denies alcohol or drug use. Manages his own medications and appointments when he has them.

PHYSICAL EXAM

General: Awake, alert, in no acute distress.

HEENT: Normocephalic. Mucous membranes moist.

Cardiovascular: Regular rate and rhythm. No murmurs.

Respiratory: Clear to auscultation bilaterally. No increased work of breathing.

Abdomen: Soft, non-tender, non-distended.

Extremities: Left lower leg with erythema, warmth and mild edema from ankle to mid-calf. Borders marked. No fluctuance, no crepitus, no drainage. Healing 2 cm abrasion over the shin. Pulses intact.

Skin: As above. Otherwise intact.

Neuro: Alert and oriented x4. No focal deficits.

Psych: Calm, cooperative. Normal mood and affect.

ASSESSMENT AND PLAN

- Left lower leg cellulitis, non-purulent: IV cefazolin, elevate, mark borders, follow cultures.
- Hypertension: resume lisinopril.
- Disposition: unhoused. Case management consult for recuperative care.

Progress Notes

08/05/2026 09:10 Priya Nandakumar, MD Hospital Medicine Progress Note

Afebrile for 24 hours. Erythema receding about 4 cm inside the marked border. Pain 3 of 10. Walking in the hallway without help. WBC 11.2, down from 13.8. Blood cultures no growth to date.

Plan: change to oral cephalexin 500 mg every 6 hours today. Watch 24 hours on oral therapy.

08/06/2026 08:45 Priya Nandakumar, MD Hospital Medicine Progress Note

Afebrile for more than 48 hours. Erythema continues to recede on oral cephalexin. No drainage. WBC 8.9. Blood cultures no growth at 48 hours. Takes his own oral antibiotic at the bedside on schedule.

Medically ready for discharge 08/07 to finish 7 more days of oral cephalexin. Needs a clean place to rest and elevate the leg. Recuperative care referral in progress.

Physical Therapy Evaluation

Brooke Tanaka, PT, DPT - 08/05/2026 13:20 - Signed

Area	Finding
Bed mobility	Independent
Transfers	Independent
Ambulation	250 ft, modified independent, no device, one standing rest break
Stairs	12 steps with rail, independent
Balance	Good static and dynamic
Assistive device	None needed

ASSESSMENT

Patient moves safely and independently. Mild antalgic gait from left leg discomfort. No loss of balance. No skilled physical therapy needs.

Discharge recommendation: No PT follow-up needed. Safe for discharge to a community setting.

Nursing Assessment / ADL Flowsheet

J. Castellanos, RN - 08/06/2026 07:30

Item	Assessment
Orientation	Alert and oriented x4
Bathing	Independent
Dressing	Independent
Toileting	Independent. Continent of bowel and bladder
Feeding	Independent
Medications	Takes own medications with RN observing. Names each one and its purpose
Fall risk	Low
Skin	Left lower leg erythema, receding. Small healing abrasion on shin. No open wound
Isolation	None
Safety screen	Suicide risk screen negative. Denies thoughts of self-harm

NURSING NOTE

Patient showers independently. Manages own hygiene. Calm and cooperative.

Vital Signs Flowsheet

Date/Time	BP	HR	RR	Temp F	SpO2	O2
08/05/2026 20:00	128/78	82	16	98.4	97%	Room air
08/06/2026 00:10	122/74	76	16	98.1	98%	Room air
08/06/2026 04:05	118/72	72	14	97.9	97%	Room air
08/06/2026 08:00	126/80	78	16	98.2	98%	Room air
08/06/2026 12:15	130/82	80	16	98.6	98%	Room air

Laboratory Results

Collected 08/06/2026 05:40

COMPLETE BLOOD COUNT

Test	Result	Reference	Units
WBC	8.9	4.0-11.0	K/uL
Hemoglobin	12.8	12.0-17.0	g/dL
Hematocrit	38.4	36-50	%
Platelets	265	150-400	K/uL

BASIC METABOLIC PANEL

Test	Result	Reference	Units
Sodium	141	135-145	mmol/L
Potassium	4.4	3.5-5.1	mmol/L
Chloride	104	98-107	mmol/L
CO2	25	22-29	mmol/L
BUN	16	7-20	mg/dL
Creatinine	1.0	0.6-1.3	mg/dL
Glucose	95	70-99	mg/dL

TRENDS AND OTHER RESULTS

Test	Collected	Result	Reference
WBC	08/03, 08/05, 08/06	13.8, 11.2, 8.9 K/uL	4.0-11.0
Lactate	08/03/2026 21:40	1.1 mmol/L	0.5-2.0
C-reactive protein	08/04/2026 05:30	48 mg/L	<10

MICROBIOLOGY

Test	Collected	Result
Blood culture x2	08/03/2026 21:45	No growth at 48 hours
MRSA nares PCR	08/04/2026 06:10	NOT DETECTED
SARS-CoV-2 antigen	08/06/2026 14:10	NEGATIVE

ELLERY, MARCUS (MRN: SYN-104417) DOB: 02/11/1972 54y M Pacific Crest Medical Center · 4 West Medical-Surgical

Diagnostic Imaging Report

Venous duplex ultrasound, left lower extremity - 08/03/2026 22:30

Indication: Leg swelling and redness. Rule out deep vein thrombosis.

FINDINGS

The common femoral, femoral, popliteal and calf veins are compressible with normal flow. Subcutaneous edema of the lower leg. No drainable fluid collection.

IMPRESSION

No deep vein thrombosis. Soft tissue edema consistent with cellulitis.

Electronically signed by A. Greenbaum, MD

Discharge Medication List

Reconciled 08/06/2026

Medication	Dose	Route	Frequency	Notes
Cephalexin	500 mg	By mouth	Every 6 hours	Through 08/13/2026
Lisinopril	10 mg	By mouth	Daily	Home medication
Acetaminophen	650 mg	By mouth	Every 6 hours as needed	For pain

30-day supply of all discharge medications will be delivered to the bedside before discharge.

Case Management / Social Work Note

Karen Lindqvist, RN, CCM - 08/06/2026 14:55

Met with patient at bedside. He has been unhoused about 14 months and sleeps outdoors. He has no safe place to rest, elevate the leg or keep it clean while he finishes antibiotics. He is agreeable to recuperative care. He takes his own medications without reminders and keeps his own appointments.

DISCHARGE PLANNING

Item	Status
Housing	Unhoused about 14 months. No shelter bed.
Disposition	Recuperative care referral faxed to Illumination Health + Home 08/06/2026.
Medications	Self-manages. 30-day supply to bedside before discharge.
Follow-up	Primary care, community clinic, 08/14/2026 10:30.
Transportation	Hospital will arrange a ride at discharge.
Health plan authorization	Submitted 08/06/2026. Pending.